INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Lenvatinib: differentiated thyroid cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00295
Version 6

Once daily, continuously

Treatment stages

01 **Continuous oral treatment**
Once daily, continuously

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	4
03	Symptoms and supportive care	10
04	Sources and further information	15

My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology urgently for fever, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink. Severe chest pain, severe breathing difficulty, collapse or a seizure: call 112/999.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

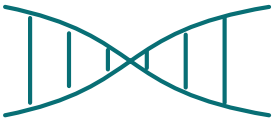
How the medicine works

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

KINASE INHIBITOR

Lenvatinib

Oral targeted treatment



Blocks tumour growth and blood-vessel signals

Lenvatinib blocks several signalling enzymes, including VEGF and FGF receptors, which support tumour blood vessels and growth. Effects on normal blood vessels explain why blood pressure, urine protein, bleeding and healing need attention.

Growth receptors blocked → Blood-vessel growth reduced → Cancer growth slowed

What to watch for

Treat diarrhoea and vomiting promptly using the agreed plan because dehydration can damage the kidneys. Check blood pressure as taught. Report sudden severe headache, vision changes, bleeding, chest pain, severe abdominal pain or fluid leakage from an abnormal opening urgently.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

What the evidence can tell you

Research supports a treatment discussion; it cannot predict one person's outcome.

SELECT randomised 392 people with radioactive-iodine-refractory differentiated thyroid cancer to lenvatinib or placebo. Median progression-free survival was 18.3 versus 3.6 months (hazard ratio 0.21; 99% CI 0.14–0.31). This endpoint is time to progression or death. The trial does not establish that an individual will live an extra 14.7 months, and its results cannot directly rank lenvatinib against sorafenib in a different trial.

CHAPTER 02

Your schedule, tests and appointments

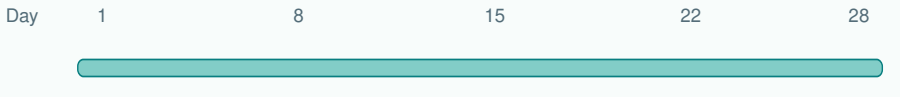
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment course

Lenvatinib: differentiated thyroid cancer

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ONCE DAILY, CONTINUOUSLY

Continuous oral treatment

Continue while helping and tolerated

Lenvatinib 24 mg once daily, continuously, until progression or unacceptable side effects. The clinician may prescribe a lower starting dose or later reductions.

The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.

BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Use the individual prescription and appointment calendar. No universal tumour-marker panel or scan interval is assumed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00295 v6: official regimen

Tests in your plan

Baseline bloods and tests

WHEN

Full blood count, kidney and liver tests, coagulation, urine protein, thyroid-stimulating hormone, ECG and blood pressure. Urine protein must be below 1 g/24 hours at initiation; blood pressure must be controlled.

Checks organ safety and suitability before treatment.

Treatment safety monitoring

WHEN

Every 28 days: blood count, kidney tests, coagulation, urine protein and thyroid-stimulating hormone. Liver tests every 2 weeks for the first 2 months, then monthly; calcium monthly. Blood pressure after week 1, every 2 weeks in the first 2 months, then monthly. ECG when clinically indicated.

Finds changes that may need supportive care, a dose hold or adjustment.

Specialist and cancer tests

If part of your plan

WHEN

Thyroid-stimulating hormone checks the hormone-suppression plan. Urine protein and blood calcium are safety checks. Thyroglobulin and response imaging follow the individual thyroid cancer plan.

Distinguishes treatment selection, hormone monitoring and cancer response.

THE WHOLE TREATMENT JOURNEY

Lenvatinib: differentiated thyroid cancer: treatment and checks

Progressive, locally advanced or metastatic differentiated thyroid cancer that is refractory to radioactive iodine.

Lenvatinib 24 mg once daily, continuously, until progression or unacceptable side effects. The clinician may prescribe a lower starting dose or later reductions.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Blood and organ safety NCCP	Full blood count, kidney and liver tests, coagulation, urine protein, thyroid-stimulating hormone, ECG and blood pressure. Urine protein must be below 1 g/24 hours at initiation; blood pressure must be controlled.	Every 28 days: blood count, kidney tests, coagulation, urine protein and thyroid-stimulating hormone. Liver tests every 2 weeks for the first 2 months, then monthly; calcium monthly. Blood pressure after week 1, every 2 weeks in the first 2 months, then monthly. ECG when clinically indicated.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Appointments and medicines NCCP	Agree the treatment aim, dose and complete medicines list.	Take at about the same time each day, with or without food. Swallow capsules whole with water; do not crush or break them. A pharmacist can teach suspension preparation: place up to 5 whole capsules in a small container or 20 mL oral syringe, add 3 mL water, apple juice or milk, wait 10 minutes, then stir or shake for 3 minutes. Take all the mixture, add another 2 mL liquid and take the rinse. For a feeding tube use water only. If a missed dose cannot be taken within 12 hours, skip it and take the next dose at the usual time.	Keep a current dose and symptom record.	Treat diarrhoea and vomiting promptly using the agreed plan because dehydration can damage the kidneys. Check blood pressure as taught. Report sudden severe headache, vision changes, bleeding, chest pain, severe abdominal pain or fluid leakage from an abnormal opening urgently.	Keep follow-up and recovery checks even after the last dose.

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Cancer-specific and specialist tests NCCP	Thyroid-stimulating hormone checks the hormone-suppression plan. Urine protein and blood calcium are safety checks. Thyroglobulin and response imaging follow the individual thyroid cancer plan.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.	Agree the named follow-up team and remaining checks.

What each check is for

1

Treatment and review

Once daily, continuously

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Ask / act: Every 28 days: blood count, kidney tests, coagulation, urine protein and thyroid-stimulating hormone. Liver tests every 2 weeks for the first 2 months, then monthly; calcium monthly. Blood pressure after week 1, every 2 weeks in the first 2 months, then monthly. ECG when clinically indicated.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before starting

Full blood count, kidney and liver tests, coagulation, urine protein, thyroid-stimulating hormone, ECG and blood pressure. Urine protein must be below 1 g/24 hours at initiation; blood pressure must be controlled.

STEP 2

Taking or receiving treatment

Take at about the same time each day, with or without food. Swallow capsules whole with water; do not crush or break them. A pharmacist can teach suspension preparation: place up to 5 whole capsules in a small container or 20 mL oral syringe, add 3 mL water, apple juice or milk, wait 10 minutes, then stir or shake for 3 minutes. Take all the mixture, add another 2 mL liquid and take the rinse. For a feeding tube use water only. If a missed dose cannot be taken within 12 hours, skip it and take the next dose at the usual time.

STEP 3

Tests and reviews

Every 28 days: blood count, kidney tests, coagulation, urine protein and thyroid-stimulating hormone. Liver tests every 2 weeks for the first 2 months, then monthly; calcium monthly. Blood pressure after week 1, every 2 weeks in the first 2 months, then monthly. ECG when clinically indicated.

STEP 5

Leaving with a clear plan

Confirm the next appointment, dose, support medicines and verified daytime and out-of-hours phone numbers.

Your exact treatment schedule

Tests and preparation

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Your continuous-treatment review window.

Timing is a general guide; symptoms may start or stop at other times.

Treatment or check	Timing in this view
Lenvatinib every day	 **Days 1–28** Lenvatinib 24 mg once daily, continuously, until progression or unacceptable side effects. The clinician may prescribe a lower starting dose or later reductions.

This 28-day window is a review calendar, not a rest cycle or a fixed treatment duration.

Treatment continues daily across review dates unless your team tells you to hold or stop. No automatic rest week is scheduled.

> Use your treating service's written temperature threshold and urgent-contact plan. Fever, shivering or sudden illness can be serious even without a high temperature. Do not wait for the next visit. Severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency help: call 112 or 999.

Read the whole review window as text

Day 1–28: Every day, including weekends

Take at about the same time each day, with or without food. Swallow capsules whole with water; do not crush or break them. A pharmacist can teach suspension preparation: place up to 5 whole capsules in a small container or 20 mL oral syringe, add 3 mL water, apple juice or milk, wait 10 minutes, then stir or shake for 3 minutes. Take all the mixture, add another 2 mL liquid and take the rinse. For a feeding tube use water only. If a missed dose cannot be taken within 12 hours, skip it and take the next dose at the usual time. Follow your prescription. A hold or restart must be agreed with the team.

Support around the medicines

1 Preventing and treating sickness

At more than 12 mg/day, lenvatinib needs the oral moderate-to-high sickness-risk plan; at 12 mg/day or less, use the minimal-to-low oral plan. The team selects support for the actual dose.

2 Medicine-specific support

3 Mouth care, nutrition and daily life

Follow the unit's mouth-care plan. Report pain, poor intake or weight loss early; ask for dietetic support. Pace activity, rest and work around symptoms.

4 Medicine and interaction review

Bring all prescription, non-prescription and herbal medicines. Check with the oncology pharmacist before starting or stopping anything.

Your diagnosis and treatment setting

Specialist tests

Thyroid-stimulating hormone checks the hormone-suppression plan. Urine protein and blood calcium are safety checks. Thyroglobulin and response imaging follow the individual thyroid cancer plan.

Treatment aims and choices

Review and alternatives

The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval. Discuss suitable alternatives, supportive care and the option of declining treatment.

Questions worth asking

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

Try explaining the plan back: Explain when you take or receive treatment.,Name the tests and next review date.,Show how to contact the team urgently.,Explain what to do if a dose is missed or treatment is held.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Lenvatinib >12 mg/day

Active medicines: Lenvatinib

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Antiemetic plan for current oral dose	Individual prescription	As agreed with oncology/pharmacy Review QT prolongation and all interactions; no automatic intravenous (IV) chemotherapy script.

Additional phase-specific support

- Take at about the same time each day, with or without food. Swallow capsules whole with water; do not crush or break them. A pharmacist can teach suspension preparation: place up to 5 whole capsules in a small container or 20 mL oral syringe, add 3 mL water, apple juice or milk, wait 10 minutes, then stir or

shake for 3 minutes. Take all the mixture, add another 2 mL liquid and take the rinse. For a feeding tube use water only. If a missed dose cannot be taken within 12 hours, skip it and take the next dose at the usual time.

- Every 28 days: blood count, kidney tests, coagulation, urine protein and thyroid-stimulating hormone. Liver tests every 2 weeks for the first 2 months, then monthly; calcium monthly. Blood pressure after week 1, every 2 weeks in the first 2 months, then monthly. electrocardiogram (ECG) when clinically indicated.
- Treat diarrhoea and vomiting promptly using the agreed plan because dehydration can damage the kidneys. Check blood pressure as taught. Report sudden severe headache, vision changes, bleeding, chest pain, severe abdominal pain or fluid leakage from an abnormal opening urgently.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Lenvatinib ≤12 mg/day

Active medicines: Lenvatinib

Emetogenic risk: Minimal to low (oral)

Medicine	Dose / route	When
Antiemetic plan for current oral dose	Individual prescription	As agreed with oncology/pharmacy Review QT prolongation and all interactions; no automatic intravenous (IV) chemotherapy script.

Additional phase-specific support

- Take at about the same time each day, with or without food. Swallow capsules whole with water; do not crush or break them. A pharmacist can teach suspension preparation: place up to 5 whole capsules in a small container or 20 mL oral syringe, add 3 mL water, apple juice or milk, wait 10 minutes, then stir or shake for 3 minutes. Take all the mixture, add another 2 mL liquid and take the rinse. For a feeding tube use water only. If a missed dose cannot be taken within 12 hours, skip it and take the next dose at the usual time.
- Every 28 days: blood count, kidney tests, coagulation, urine protein and thyroid-stimulating hormone. Liver tests every 2 weeks for the first 2 months, then monthly; calcium monthly. Blood pressure after week 1, every 2 weeks in the first 2 months, then monthly. electrocardiogram (ECG) when clinically indicated.
- Treat diarrhoea and vomiting promptly using the agreed plan because dehydration can damage the kidneys. Check blood pressure as taught. Report sudden severe headache, vision changes, bleeding, chest pain, severe abdominal pain or fluid leakage from an abnormal opening urgently.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Fatigue and neurological symptoms

Lenvatinib

Balance activity and rest. Sudden confusion, seizure, severe headache or a change in vision needs urgent help.

2 Mouth, eating and swallowing

Lenvatinib

Report ulcers, pain, dry mouth, difficulty swallowing or weight loss early. Ask for oral care, dietetic and swallowing support.

3 Heart and blood pressure

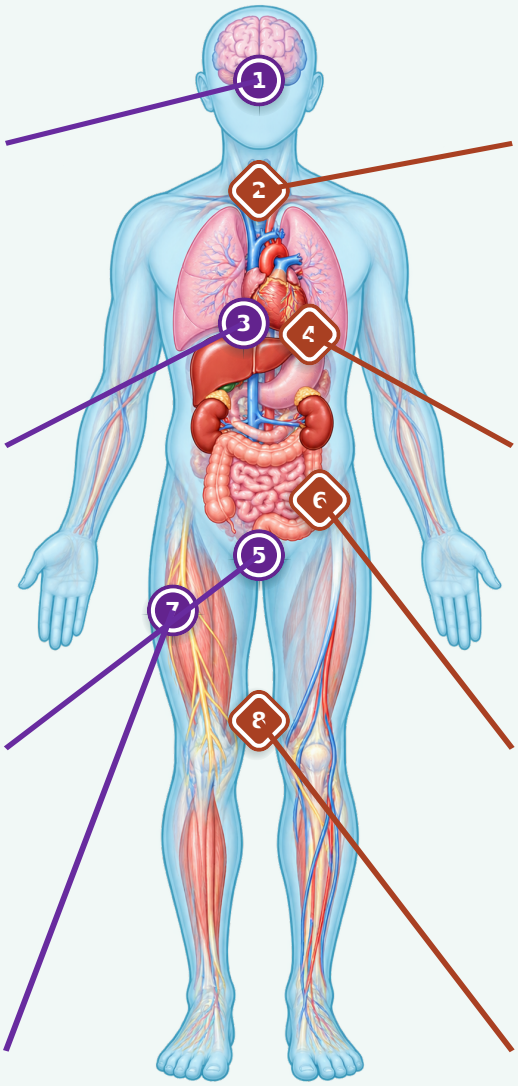
Lenvatinib

Follow prescribed heart and blood-pressure checks. Chest pain, fainting, palpitations or new breathlessness needs urgent advice.

4 Breathing

Lenvatinib

New cough or breathlessness needs prompt review; serious causes include infection or lung injury.



5 Sickness and bowels

Lenvatinib

Use prescribed support. Report persistent diarrhoea or vomiting, inability to drink, blood or severe abdominal pain.

6 Kidneys and fluid balance

Lenvatinib

Report reduced urine, swelling or poor fluid intake; keep blood and urine tests.

7 Skin, hands and feet

Lenvatinib

Report painful palms or soles, rash or blistering early. Severe spreading rash or mouth/eye involvement needs urgent review.

8 Fertility and pregnancy

Lenvatinib

Discuss fertility preservation, contraception, pregnancy and breastfeeding before starting.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology urgently for fever, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink. Severe chest pain, severe breathing difficulty, collapse or a seizure: call 112/999.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00295 v6: official regimen**
   https://healthservice.hse.ie/documents/6951/295_V6_lenvatinib-DTC_therapy.pdf
2. **Lenvima: EMA product information, including trial evidence**
   https://www.ema.europa.eu/en/documents/product-information/lenvima-epar-product-information_en.pdf
3. **NCI: oral complications of cancer therapies, patient version**
   https://www.cancer.gov/about-cancer/treatment/side-effects/mouth-throat/oral-complications-pdq
4. **NCI: nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
5. **HSE: cancer treatment side effects**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
6. **NCCP supportive-care antiemetics for NCIS**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf